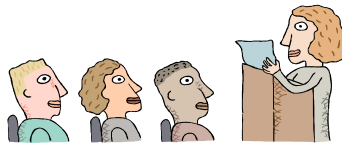


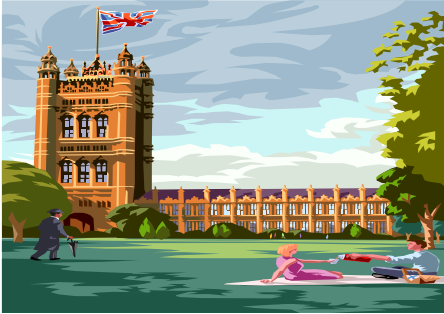
**Marion Isaacs D'Lugoff MA, RN-CS**  
**Clinical Director, Wald Community Nursing Center &**  
**Assistant Professor, Johns Hopkins University School of Nursing**

Dinner Presentation April 2, 2003  
American Association of Colleges of Nursing/  
National Health Service Corps Ambassador's Annual Meeting



# Lillian D. Wald Community Nursing Center

A Service of the Johns Hopkins University  
School of Nursing



The building on the left is a slightly idealized version of our clinic at the Rutland Center (if you think it looks more like Cambridge University than a recycled, city elementary school, you are correct. The Rutland Center is currently home to 37 families living in a transitional shelter, an early Head Start, daycare center, church and church run wellness center. Pictured on the right is the actual view from our window.

## Learner Objectives....



**Enjoy  
your patients**

**Laugh at  
their jokes.**

**Laugh  
period.**

**Applaud their  
strengths and  
successes,  
literally.**



## Learner Objectives:

After this presentation the participant will understand the value of enjoying the patient, etc. My patients are funny! I try to be funny too. My patients are considerate they laugh at my jokes even when they aren't fine. Having a good time is contagious. Work becomes pleasurable and satisfying. The word courage forms the root of the word "encourage" encouraging patients gives them support, and affirmation for their strengths and their accomplishments.

Recognize how difficult behavior change is- Applaud every day that someone has given up drugs, cigarettes, ice cream sundaes (my personal high), etc.

Hugging is a cultural norm in our community. I always hugged one of my clients, a pastor and foster parent to 6 children. I thought this hug expressed my respect and admiration for the wonderful deed she showed in lovingly, raising these children. Then I realized that she was hugging me! Maybe she thought I deserved it or maybe she thought I looked tired, who knows? I know I enjoyed it.



In the shadow of Hopkins Medical campus is the poorest, most morbid population in Baltimore city. The health indices, infant mortality and so forth parallel that of 3<sup>rd</sup> world countries. Not even the most sophisticated medical care and research can counter the effects of environmental neglect, and poverty. The Johns Hopkins University School of Nursing has made a significant investment in reaching out to its neighbors.

## About the Program

The Johns Hopkins University School of Nursing, Lillian D. Wald Community Nursing Center (Wald) is one of a few health programs in Baltimore City providing, nearly barrier-free, health services to poor, uninsured or under insured residents of Baltimore City at no charge to them.

The program is supported by foundations, grants and the School of Nursing. All program employees are staff, faculty and students of the School of Nursing. Volunteer medical consultants are associate faculty members.

Who we are

## Mission

To improve the health of the Baltimore community through the provision of quality nursing services for uninsured and under-insured individuals and families.

To foster health promotion through provision of primary and secondary prevention activities with advocacy, referral, and follow-up to access comprehensive primary care and tertiary services

## Vision

To enhance the vitality of the community through improvements in the quality of life and health of the residents of Baltimore City that can be achieved by closing the gap to accessible, acceptable, and available health care for poor families.

Why we are

## POVERTY IN AMERICA

### DEFINITION OF POVERTY

|                                        |            |
|----------------------------------------|------------|
| Family of four                         | \$17,000   |
| Number of poor                         | 35 million |
| Poverty percentage of total population | 12.7%      |



### RACE (% poor)

|          |       |
|----------|-------|
| White    | 10.5% |
| Black    | 26.1% |
| Hispanic | 25.6% |

### AGE (% poor)

Under 18  
Over 65



### FAMILY COMPOSITION (% poor)

Married couple

Female householder, no husband  
Female householder, no husband, children under 18



Dye, Thomas Understanding Public Policy, Prentice-Hall, 2000. Census

Like many of you we work with very, poor families. This factor is critical in correctly assessing patient's needs and putting interventions into a context that will improve the chance of success. From the slide above it is evident that the female-headed family with young children is the poorest group in America (50.5% of the population). By comparison, the aged, represent only 10.5% of the poor in America. The existence of social programs supporting the elderly is one explanation for this difference.

# 32 Million Americans Are Hungry

4 million children under 12 in the U.S. are hungry.  
An additional 9.6 million are at risk of hunger.

25% children under six live in poverty.



20% Children Under 18 Years Of Age Are Poor.

This Is Double The Child Poverty Rate  
Of Any Other Industrial Country.

Even short periods of under-nutrition can affect a child's behavior, cognitive development, and future productivity.

Based on Dept of Agriculture & poverty estimates.



Paying for housing competes with paying for food. Families that live in subsidized housing have less iron deficiency anemia than families that live in the same neighborhood but pay market value for rent.

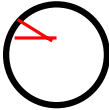
**1 in 10 American families is at risk of going hungry**



**Most families with a hungry child  
have a least 1 parent working full-time**



**2.5% of the federal budget is spending for food.**



**25% of all food produced  
in the U.S. is wasted.**



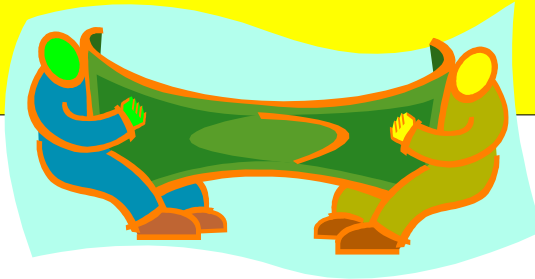
**USDA provides food stamps to about 18 million low-income families  
and individuals and school lunches to more than 26 million children.**



Political awareness has been a constant aspect of public health. Encouraging practitioners to become involved in the political process accomplishes several things: It directs anger into action; it models behavior that builds community capacity; it leads to change.

Professional medical organization political action led to the creation of the school lunch program, the Reach Out and Read Program, etc.

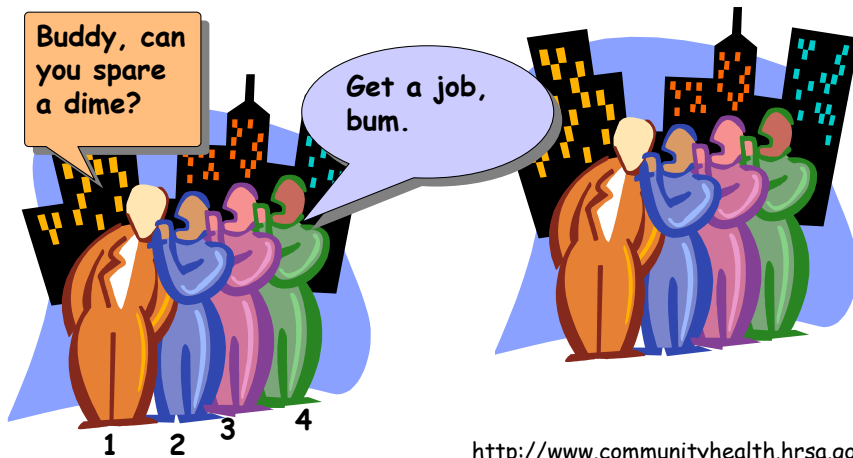
*"Baltimore is an older, industrial city with a wide variety of housing and community development needs. Its most basic problem is that so many of its residents are poor."*



**U.S. Department of Housing and Urban Development**  
Office of Community Planning and Development

Baltimore City African-American. 65 %  
White 30 %  
Hispanic, other 1-5 %

24% of people living below the poverty level



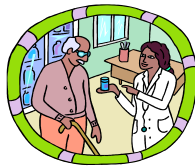
<http://www.communityhealth.hrsa.gov/countyinfo>

The uneducated, perhaps knee-jerk response to unemployment is often:  
“Get a Job”.

## **Jobs Jobs Jobs Jobs Jobs Jobs**

**36% decrease in manufacturing jobs.  
Bethlehem Steel.**

**60,000 jobs were lost in  
Baltimore from 1989-1995**



Where? Poverty is inextricably tied to employment. Industrial jobs (unionized, good pay, good benefits) have been replaced with low-paying fast food jobs without health and other benefits. Many health care jobs (nursing assistants in nursing homes for example don't include health benefits— How crazy making is it to insist on providing compassionate health care to the elderly, when the worker has none.

**Minimum Wage  
Raised to \$5.15  
(9/1/1997)**

$$\begin{array}{r} \$5.15 \times \\ 40 \text{ hrs} \times \\ \hline 52 \text{ wks} = \end{array}$$

**\$10,712**

$$\text{\$10,712/12} = \text{\$892/4} = \text{\$223.}$$

**\$321 = Median monthly rent  
in the city**

**Federal Poverty Guidelines**

| Family<br>Size | Annual Income.<br>100% |
|----------------|------------------------|
| 2              | \$11,250               |
| 3              | \$14,062               |
| 4              | \$17,050               |
| 5              | \$19,950               |
| 6              | \$22,850               |

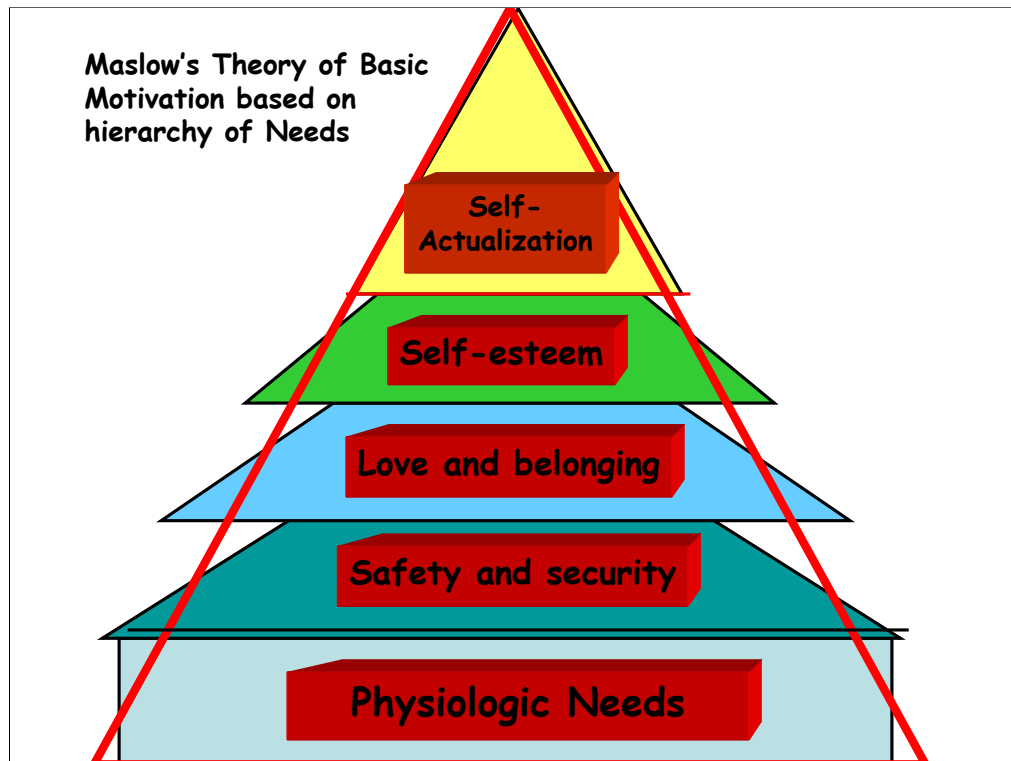
**24% of the City of Baltimore lives below  
the poverty level**

Minimum wage results in income below the poverty level.

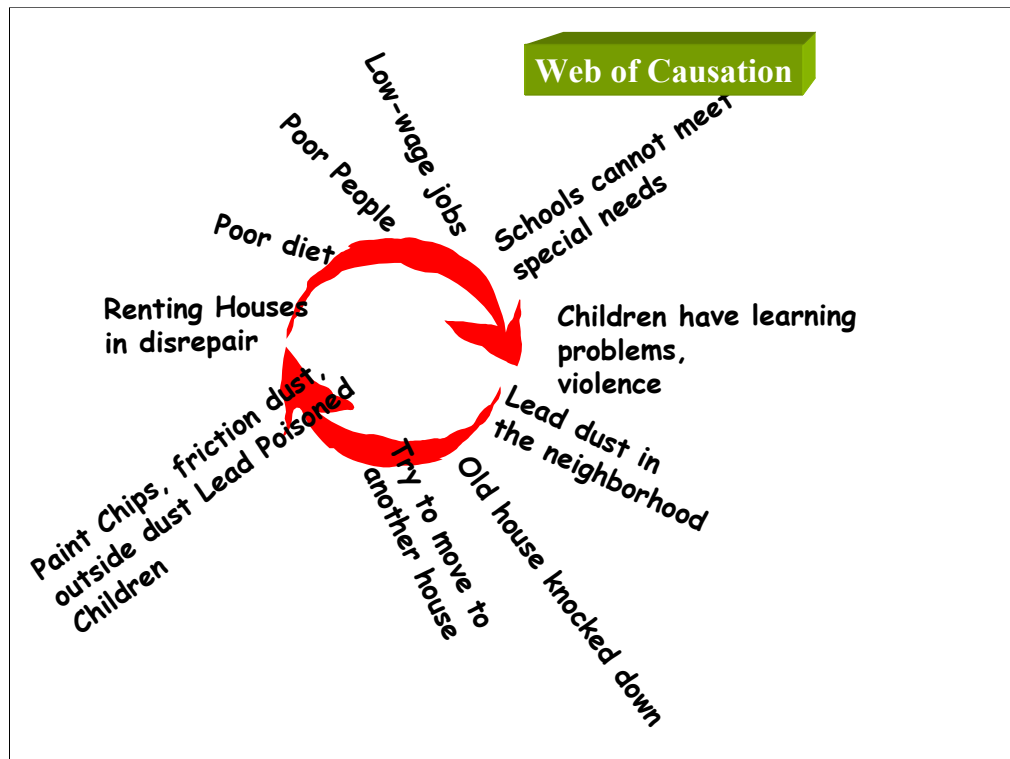
Minimum wage doesn't pay the rent in Baltimore city monthly Weekly gross \$223. Monthly rent \$321. Families are precariously housed, moving as often as 5 times a year from overcrowded family situations, to shelters. Public housing subsidies have declined steadily since the 1980's.



The housing that is affordable is often next to abandoned housing, crack houses, or houses that have been used to illegally dump trash (Vacancies, danger, despair). Some people live in condemned housing, with no water no heat.



The principle impact of poverty is that it is a barrier to individual, family and community progress. Poverty limits options. Human beings have difficulty being creative, productive, community minded people when their own physiologic needs for housing, food, water etc are inadequately met.



An example of the Web of causation applied to lead poisoning. Starts with poor people...

## Barriers that prevent or limit access of the insured:

1. **Temporal barriers**-long waits for appointments, long waits at appointments, multiple personal, family and community crises impinging on appointment planning and keeping;



2. **Geographic barriers** -lack of transportation, frequent change of address, inability to dis-enroll from MCOs which are at a distance from the current residence;



Patients often complain that they have to wait hours to be seen.

Somehow it seems basically unfair to deny a patient's visit if they are 15 or 20 minutes late for their appointment.

Policies such as this:

creates a system of contempt for the struggles the patient must overcome to get to that appointment.

impedes the mission of the organization to provide the health services,

subverts the public health outcomes that health insurance for the poor seeks to improve.

**3. Intellectual barriers** inability to negotiate the system, poor communication with the ancillary and professional staff (the former described as "having an attitude", the latter "incomprehensible").



**4. Psychological barriers -**  
fear, distrust,  
and inexperience with medical interventions



Opportunity overcomes hopelessness.

I am lucky enough to have worked long enough with the same population to see enormous growth and change in people.

"Pam" and her 3 children were homeless in 1992. Now, she is working, housed and her oldest daughter is attending college and in the United States Army.



Substance abuse and domestic violence to major problems in Baltimore  
1 in 4 = 65,000 addicts 5,000 treatment slots.



Let your staff, your community and your patients know how important your work is. Make your work high profile- bring prestige to the program- share your enthusiasm. Everyone benefits= Staff feel recognized, patients feel like they are giving back. Maybe you will get more things.

Pictured are Mayoral and City Council Proclamations, we solicited for our 10<sup>th</sup> anniversary.



Lieutenant Governor, Kathleen Kennedy Townsend visits to announce the expanded MCHIP program. We all love the attention!



Mark Gearin, former head of the Peace Corps visited. We brought him upstairs to see a shelter apartment. It doesn't hurt to share publicity with your partner community agencies.



Carol Long, PNP

This nurse practitioner volunteered for a year before salary support was available for her position!!! Creating a missionary zeal validates your best intentions and is contagious.



Nancy Glass, PhD, MS, RN

Bring talented like minded people into the fold and nurture them- Nancy Glass was a 1992 undergraduate when she started at Hopkins as a student. She joined the faculty, completed her PhD. Completed meaningful research in domestic violence and replicated and improved on our model at another university school of nursing.



Eric Levey, MD  
Pediatrics

Our consulting pediatrician is on the faculty of Hopkins and has appointments at the Kennedy Krieger Institute and at the Hopkin's Harriet Lane Clinic..



J. Courtland Robinson, MD  
Hopkins  
OB-GYN

Our OB-GYN consult is a former medical missionary in Thailand; head Planned parenthood; head of OB-GYN.



Philip Zieve, MD  
Vice President  
Hopkins Bayview  
Internal Medicine

Our consult in internal medicine authored Principles of Ambulatory Care, Best selling text in ambulatory health care.



This Advisory board member volunteered at our clinic and orchestrated a fundraiser that brought in \$100,000.



The client on the left is the sister of Linda Whitner. Linda Whitner is the most resourceful networker. She encourages one patient to help another.

Linda Whitner, our community health worker, once lived in Transitional housing. She role models courage, success and responsibility every day!



We relish the spirit of diversity that we nurture in our students and the barriers that this break down.



"Uduak" came from Nigeria, with her father and 3 siblings when she was 17 years old, her mother stayed behind. Shirley van Zandt is her nurse practitioner. She touches base often with Linda Whitner, CHW for advice, shopping trips and other mother/daughter experiences. This is her high school prom picture. She is currently a junior living on campus at Coppin State College.

Immigrants- Often uninsured, arrive with unique cultural and medical problems. What a great opportunity for us learn about the implications of global health problems in our own backyard and to eat new and Delicious foods brought in for us by these families.



Mary Faltinsky, is pictured here as a nursing student with some of the children attending the class she taught called, Wiping 101.

Making it real and making it fun works!

# RESPECT

Welcome the patient.

Introduce your self.

Shake hands.

Make the patient comfortable.

Welcome the patient back....

Dress nicely.

Share food.



Have fun, enjoy yourself.

Don't build artificial boundaries in the name of professionalism

This is a page from our Orientation Manual.



Respect- even in shots and blood draws. See how this 2 year old generously gives us her arm to draw a blood lead level.

If we hurt a child in the course of treatment it is de rigueur to whisper an apology into the child's ear. Even 2 year olds seem to sense our concern when we do this.



*Lillian D. Wald, about 1915.*

# Lillian D. Wald

## 1867-1940

Working with poor people, in their communities is nothing new to us we have been doing it for nearly 100 years